[redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927 [redacted]

DOA 17/Feb/2026 00:43

[redacted]

BED No. ICU - 1 DIET

[redacted]

DATE : 17/02/26 DOA : 17/02/26

CONSULTANT : [redacted] Balance

CURRENT DIAGNOSIS ANEMIA

HBS Ag / HIV / HCV

Allergy

SPECIAL INSTRUCTION
- Chest X RAY PA view due
- 2D echo (Ask)
- USG NIA
- Iron Profile

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- I/O CHARTING
- 2 O PRBC arranged

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
	M			E				N																			
INJ. MONOCEF	1GM	12HRLY	IV																		1:30	[redacted]					
INJ ACILOC	50MG	12HRLY	IV																		✓					⊙	
INJ CEMSET	4MG	12HRLY	IV																		✓					⊙	
INJ. TRANEXA	1GM	SOS	IV																								
INJ. LASIX	20MG	SOS	IV																								
INJ. MVI	1AMP	24HRLY	IV																								
TAB. DROSOME	1TAB	24HRLY	P/O																								
																					(1:15)						
BP																					114/67	110/70	130/70	130/80	140/80	140/80	130/80
CVP																					–	–					
TEMP																					98.2F	98.2F					
PULSE																					95	96	97	98	97	97	98
SPO_2																					99%	98%	96	97	97	96	97
RESP																					18	20	18	16	17	16	18
BLOOD SUGAR																					NS(500ml)						
INTAKE IV																										H50(100ml)	
ORAL/RT																											
OUTPUT																											
URINE																							550 ml				
STOOL																											
VOMITTING																											

GCS/TIME	8	12	16	20	24	4
SENSORIUM						Conf
EYE RESPONSE						4
VERBAL RESPONSE						5
MOTOR RESPONSE						6
PUPIL SIZE						
RIGHT						N
LEFT						N
VOMITTING						No
Pain Score						2/10
Back Care						✓
VENTILATOR						
TV						
PRES						
O_2FIO_2						
PEEP						
RATE						
PEEK PRES						
ABG						
Ph						
PO_2						
PCO						
HCO_3						
BE						

INVESTIGATION	STAT ORDER	BY
CBC, RFT, LFT Blood GP., HBA1C, CPK / CPK MB ⊗ due. - Stool for occult Blood due.		

Total Intake ..... Ltr. Output ..... Ltr. Balance ..... Ltr.

Remarks Nurse 1 ..... 2 ..... 3 ..... [redacted] RMO 1 ..... RMO 2 ..... RMO 3 ..... [redacted] .....

EVENT .....

PTO FOR NOTES

14/2/26

1Am

8Am

NIGHT DUTY NOTES

Patient recieved from ENR staff
Vitals are stable & documented
Intake output charting done.
All prescribed medication done.
Patient is stable & conscious
2O PRBC arranged.. No
fresh complaint till now.

[redacted]

[redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

[redacted]

ART

BED No. ICU-1 DIET

[redacted]

DATE: 17/2/26 DOA: 17/2/26

CONSULTANT: [redacted]

Balance

CURRENT DIAGNOSIS: ANEMIA, HTN, DM, CAD c PTCA

HBS Ag / HIV / HCV

Allergy: N.K

SPECIAL INSTRUCTION
- Chest x-Ray PA view due
- 2D echo, USG WA (ASK)
- Iron Profile (ASK)
- Stool for occult Blood (ASK)

PROCEDURE DATE: CAG, PTCA, PA line, Venti, IABP, CABG

INSTRUCTIONS
- MONITOR VITALS
- S/O CHARTING

* 2 ⊙ PRBC arranged.

MEDICATION	DOSE	FREQ	ROUTE	M 8	E 9	10	11	12	13	14	N 15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
INJ. MONOCEF	1GM	12HRLY	IV													[redacted]											
INJ. ACILOC	50MG	12HRLY	IV																							[redacted]	
INJ. EMSET	4MG	12HRLY	IV																								
INJ. TRANEXA	1GM	SOS	IV	Stop																							
INJ. LASIX	20MG	SOS	IV																								
INJ. MVI	1AMP	24HRLY	IV																								
TAB. GRIDSOME	1TAB	24HRLY	PO																								
INJ AVIL	1AMP	SOS	IV																								

GCS/TIME	8	12	16	20	24
SENSORIUM	Conf	Conf	Cons	Cons	Cons
EYE RESPONSE	4	4	4	4	4
VERBAL RESPONSE	5	5	5	5	5
MOTOR RESPONSE	6	6	6	6	6
PUPIL SIZE					
RIGHT	N	N	N	N	N
LEFT	N	N	N	N	N
VOMITTING	No	No	No	No	No

	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
BP	130/90	120/80	110/70	120/76	120/70	120/76	120/70	120/80	130/80	120/90	120/80	120/80	120/78	120/70	125/75	110/70	120/65	130/90	120/70	120/70	124/74	100/75	108/76	120/70
CVP																								
TEMP	98F	98F	–	–	–	–	99.8F	98.2F		98.6F	–	–	98.2F	–	–	98.2F	–	–	–	–	–	98.1F	–	–
PULSE	98/min	90	88	180	180	80	98/min	90	89/min	80	80	80	82	86	88	88	80	78	80	82	84	80	80	86
SPO_2	97%	97%	97%	85%	97	98	94%	95	96	98	98	98	98%	99%	98%	97%	99	97	98	99	98	97%	98%	99%
RESP	18	16	14	16	16	18	16	18	18	19	19	19	20/min	20/min	18/min	18/min	18/min	16	14	16	14	18	18	20

	8	12	16	20	24
VENTILATOR					
TV					
PRES					
O_2FIO_2					
PEEP					
RATE					
PEEK PRES					
ABG					
Ph					
PO_2					
PCO					
HCO_3					
BE					

BLOOD SUGAR: 172 mg/dl

INTAKE IV

ORAL/RT: BV 250ml, Tea 100ml; Tea 50; Juice 200ml; Milk (200ml); Water (100ml); Tea (100ml)

OUTPUT

URINE: 200ml; 250ml; 150ml; 300ml

STOOL

VOMITTING

INVESTIGATION	STAT ORDER	BY
LBC, RFT, LFT, Blood GP, HbA1c, CPK/CPK-MB (R) due.	Stool for occult	
Trop-T done	- LBC (R) done.	
1 PRBC	Rundown	
1 PRBC	—	

Total Intake 650 ml Ltr. Output 550 ml Ltr. Balance Ltr.

Remarks Nurse 1 [illegible] 2 Aditya 3 Sugandha RMO 1 RMO 2 Dr. Basu RMO 3 Dr Basu

EVENT

PTO FOR NOTES

(m) Duty over

Patient Received in

(n) Duty nursing

staff assessed the pt

Condition vital

checked and Recd

(m) Duty medicine done

Chest xray, 2 D echo, USG - UA

Due Next Duty nursing

staff over

Duty nursing staff

1 PRBC Running -

on Pralet

8 AM TO 2 PM

17/2/26

[redacted]

EVENING NOTES

PT. Received from (m) dr

Pt. alert conscious

vital checked and

All medicine given for doctor order

I/O charting maintain

No IV fluid

TROP - I (-ve)

1 PRBC - T 8 am 5 pm - 5 pm

USG WA - done

2D ECHO done } BP - dry, mellitus

Stool for occult blood

2 PRBC - arrange

[redacted]

17/2/26 NIGHT DUTY NOTES

8 pm Patient handover received from evening shift staff. vitals are monitored & documented. Intake output charting done. 10 PRBC done & 10 PRBC due. USG WA, serum Profile, 2.D echo - Ask to dr. malik. Stool for occult Blood due. No fresh complaint till now. 10 PRBC is on flow.

8 am

[redacted]

ICU TREATMENT CHART

BED No. ICU-1 DIET Normal Diet (Diabetic)

[redacted]

CONSULTANT: [redacted]

PATIENT NAME [redacted]
ID 85/0927 AGE / SEX 71/F
DATE: 18/2/26 DOA: 17/2/26

Balance

CURRENT DIAGNOSIS ANEMIA
HTN | T2DM | CAD & PTCA

HBS Ag / HIV / HCV

Allergy GNJ. MNI

SPECIAL INSTRUCTION
- 1 O PRBC done on 17/2/26
- 1 O PRBC due

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- 9/0 CHARTING

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
GNJ. MONOCEF	1GM	12HRLY	IIV	Sugandha																							
GNJ. ACILOC	50mg	12HRLY	IIV											Lalita		Lalita											
GNJ. GAMSET	4MG	12HRLY	IIV											Lalita												Sugandha	
GNJ. LASIX	40MG	SOS	IIV																								
TAB. GROSOME	1TAB	24HRLY	P/O																								
TAB METFORMIN	500	OD	P/O													Sugandha											

GCS/TIME	8	12	16	20	24	4
SENSORIUM	Con	Con	Con	Con	Con	Con
EYE RESPONSE	4	4	4	4	4	4
VERBAL RESPONSE	5	5	5	5	5	5
MOTOR RESPONSE	6	6	6	6	6	6
PUPIL SIZE						
RIGHT	N	N	N	N	N	N
LEFT	N	N	N	N	N	N
VOMITTING	No	No	No	No	No	No
Pain Score	2/10	2/10	2/10	1/10	2/10	2/10
Back Care	✓	✓	✓	✓	✓	✓
VENTILATOR						
TV						
PRES						
O_2FIO_2						
PEEP						
RATE						
PEEK PRES						
ABG						
Ph						
PO_2						
PCO						
HCO_3						
BE						
	1	1	1	1		

	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
BP	120/80	130/80	120/70	120/70	120/70	120/76	120/70	120/70	120/60	130/70	130/80	130/60	130/60	124/68	125/70	120/75	120/68	124/66	130/75	132/76	125/72	130/62	135/69	130/80
CVP																								
TEMP	100.4F	99.6F	98.4F	98.4F	98.4F	—		—	98.4F	—	—	—	—	—	98.2F	—	—	—	—	—	—	—	98.1F	—
PULSE	98bpm	96bpm	90bpm	94	88	84	86	86	86	84	84	84	84	82	84	80	82	80	84	86	80	82	82	84
SPO_2	97%	97%	98%	97%	98%	97%	98	97	98	98	98	98	98	97%	98%	97%	97%	98%	97%	96%	97%	98%	99%	99%
RESP	18	18	16	16	18	16	20	18	18	20	20	20	20	18	18	20	20	18	18	22	20	20	20	22
BLOOD SUGAR																								
INTAKE IV	NS 100ml	7:30am PRBC (330ml)			[redacted]								NS(100ml) Pinnacle PRBC (240ml)		Milk (100ml)								NS(100ml) Tea	
ORAL/RT					300ml	200ml	Tea 100ml													8:20 (100ml)			(50ml)	
OUTPUT																								
URINE				300ml									450ml					250ml						400ml
STOOL																								
VOMITTING																								

INVESTIGATION	STAT ORDER	BY
- 2D echo - USG W/A - Iron Profile } Ask	CBC S.Folate Vit B12 Iron profile	dm 19/2
- Stool for occult Blood (3) due.		

Total Intake 1637 ml Ltr. Output 1100 ml Ltr. Balance 537ml (+ve Balance) Ltr.

Remarks [redacted]

VENT

PTO FOR NOTES

(M) Duty OVER

Patient Received in (M) Duty Nursing STAFF Assess the Patient Condition vital checked and Recorded (M) Duty Medicine done PRBC II Unit Hold 1A fever 8AM 100 F° given in PCM 1Gm IV STAT Next Duty Nursing STAFF OVER TO (E) Duty

Sri Bala
8AM TO 2PM
18/2/26

EVENING

Pt Received from (M) duty Staff. Pt vital checked and Received Pt ~~RBS~~ I/O Chart
II PRBC done. S/D Allowed all medical diet
No fresh complaint
all ok
BT & Hb at 1:30pm stop 2:15pm

[redacted]

18/2/26 NIGHT DUTY NOTES

8pm Patient handover received from evening duty staff. Vitals are monitored & documented. Intake output charting done. All Prescribed medication done.

8am 2 O PRBC done. Patient is taking Diabetic Normal Diet. Stool for occult Blood (S) due. No fresh complaint till now.

[redacted]

[redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43

CHART

BED No. 9W-1 DIET Diabetic Normal diet

[redacted]

DATE: 19/2/26 DOA: 17/2/26

Balance

M E N

CURRENT DIAGNOSIS ANEMIA / HTN / T2DM / CAD c PTCA

HBS Ag / HIV / HCV

Allergy c INJ. MVI

SPECIAL INSTRUCTION
- 2D Echo
- USG WA } Ask

PROCEDURE DATE
CAG
PTCA
PA line
Venti
IABP
CABG

INSTRUCTIONS
- MONITOR VITALS
- I/O CHARTING

MEDICATION	DOSE	FREQ	ROUTE	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	1	2	3	4	5	6	7
23 INJ MONOCEF	1gm	12HRLY	IV	✓ Sugandha												•											
INJ. ACILOC	50mg	12HRLY	IV		✓									•												•	
INJ. EMSET	4mg	12HRLY	IV											•												•	
INJ. LASIX	20mg	SOS	IV																								
TAB. DROSDME	1 TAB	24HRLY	P/O			✓																					
TAB. METFORMIN	500mg	24HRLY	P/O	✓ Sugandha																							
BP				130/70	120/70	[redacted]		110/80	120/80	110/70	120/80																
CVP				—																							
TEMP				98.4	98.4			98.4	98.4	98.4	98.4																
PULSE				88	80																						
SPO_2				97%	97%			97	98	97	98																
RESP				18	16			18	19	16																	
BLOOD SUGAR																											
INTAKE IV				NS 100ml						Lunch 1120																	
ORAL/RT										Lunch																	
OUTPUT																											
URINE					✓																						
STOOL																											
VOMITTING																											

GCS/TIME	8	12	16	20	2
SENSORIUM	Con				
EYE RESPONSE					
VERBAL RESPONSE					
MOTOR RESPONSE					
PUPIL SIZE					
RIGHT					
LEFT					
VOMITTING					
Pain Score					
Back Care					
VENTILATOR					
TV					
PRES					
O_2FIO_2					
PEEP					
RATE					
PEEK PRES					
ABG					
Ph					
PO_2					
PCO					
HCO_3					
BE					

INVESTIGATION	STAT ORDER
- ~~Stool for occult Blood~~ due (R) due ~~CBC, Vit B12, Serum~~ Profile, S. Folate (R) due.	

Total Intake 1580 ml Ltr. Output 1400 ml Ltr. Balance 180 ml (+ve) Ltr.

Remarks Nurse 1 [redacted] RMO 2 ..... RMO 3 .....

EVENT .....

PTO FOR

(m) Duty OVER

Patient Received to
(n) Duty nursing staff
Assess the patient
Condition vital checked
and Recorded
(m) Duty medicine done
Due lab (P)
Stool for occult
Blood (S)
2PRBC done.
Next Duty nursing
STAFF OVER
(E) Duty nursing
STAFF RNS one
Time checking
Next Duty
[redacted]
8AM TO 2hs
19/2/26

19/2/26
Evening duty reports
2pm-8pm

- Received from (E) duty staff at 2pm from Both amply
- Pt's Condition and vitals GC, BP
- All drs medicine given data added
- 2PRBC given
- Today CBC, S. Iron, S. Folate, VitB12 (N) done
- Stool for occult Blood (P) collected (negative)
- Pt no fresh complaints
- Pt's is [illegible]